Emergency Department Patient Identification

Synthetic educational example | Not clinical, legal, or operational guidance

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Owner: Emergency Department Quality Team

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Purpose

This fictional example illustrates why a healthcare workflow needs a consistent patient-identification process. It is not a substitute for an approved clinical protocol.

Identification before care

Before a clinical intervention, specimen collection, medication administration, or transfer, staff should follow the facility's approved method for confirming identity using the required independent identifiers. A room number or physical location alone should not be treated as a reliable identifier. If information conflicts or identity is uncertain, pause the workflow when safe and escalate through the designated clinical process.

Specimens and handoffs

Specimens should be labeled and verified according to the facility's procedure at the point of collection. During handoffs, staff should communicate unresolved identity discrepancies explicitly and document corrective action in the approved record. Do not guess or silently overwrite conflicting identity information.

Reporting and review

Near misses and identification errors should be reported through the facility's quality and safety process. The quality team should review patterns and update training or procedures when needed.

Training and acknowledgment

Assigned staff must read the currently approved policy and record acknowledgment. Acknowledgment is version-specific and does not replace practical training or supervision.

Search terms

patient identification, two identifiers, specimen labeling, emergency department, handoff, patient safety, identity discrepancy